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Clinic; and Doe 6 at Women’s Clinic. For purposes of the analysis that follows, I assume that Doe 1 could not get privileges.⁹ If we also assume that none of these doctors would be replaced if they ceased to perform abortions, the impact of the challenged law on abortion access in the State depends on the ability of four doctors to secure such privileges: Doe 2 (June Medical, Shreveport), Doe 3 (June Medical, Shreveport), Doe 5 (Delta Clinic, Baton Rouge, and Women’s Clinic, New Orleans), and Doe 6 (Women’s Clinic, New Orleans). As I will show, under the correct legal standard, June Medical failed to prove that Act 620 would drive these four doctors out of the abortion practice.

Doe 2. The District Court concluded that Doe 2 made a good-faith effort to obtain privileges, and the Court now affirms that holding. *Ante*, at 331. It is painfully obvious, however, that Doe 2 did not act in the way one would expect if compliance with Act 620 had been to his benefit.

E-mails in the record reveal that Doe 2 only half-heartedly applied for privileges, did so on the advice of counsel, and calculated that an outright denial would be best for his legal challenge. See App. 1452 (“The lawyers think it is important that I at least have an application pending at a hospital”); *id.*, at 1453 (“It may, however, be more important from

⁹The Fifth Circuit concluded that it would be “nearly impossible” for Doe 1 to get privileges, *June Medical Services L. L. C. v. Gee*, 905 F. 3d 787, 812 (2018), and for this reason, the plurality does not linger on Doe 1. *Ante*, at 328. Under the correct legal standard, however, it is not at all clear that Doe 1 made the effort required, at least with respect to Christus Health in Shreveport. He applied there for courtesy privileges, received letters instructing him to pick up a badge, and when he called to clarify the meaning of letters sent to him, an unnamed doctor supposedly told him that he should apply for “some kind of a nonstaff caregiver type” position, App. 725, and he then ceased all efforts to get courtesy staff privileges at Christus, *id.*, at 728. A person with a strong personal incentive to obtain courtesy privileges would not necessarily have taken this somewhat cryptic advice as a definite rejection of his application.

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a legal challenge standpoint against this Bill just to have an application pending *or even denied*” (emphasis added)).

Consistent with this attitude, Doe 2 declined to apply for privileges at a Shreveport-area hospital, Christus Health, where he previously had privileges while performing abortions offsite and where another doctor who performed abortions, Doe 3, maintained privileges. *Id.*, at 382. Doe 2 knew that Doe 3 had privileges at Christus Health, a hospital that grants “courtesy privileges,” which allow doctors to admit patients but do not require a minimum number of admissions. See *id.*, at 406; Record 12125 (bylaws).

Doe 2’s stated reasons for not applying to Christus Health are not reasons that are likely to have deterred an individual with a strong personal incentive to obtain privileges. He testified that Christus is a Catholic hospital and that he did not apply there for that reason. App. 405–406. He added that he applied to other hospitals where he “knew people and might feel more comfortable,” “places that [he] thought meant something” and where he would have “the highest likelihood” of obtaining privileges. *Id.*, at 454. A person with a strong personal incentive to get privileges is not likely to have found these reasons sufficient to justify failing even to apply.

The District Court did not address Doe 2’s failure to apply to Christus Health. 250 F. Supp. 3d, at 68–74. The plurality, however, argues that Christus would not have granted Doe 2 privileges because its bylaws object to abortion practice. *Ante*, at 330–331. But as noted, Christus Health had previously granted privileges to doctors who perform abortions. Not only did Doe 2 have privileges there while he was performing abortions, but Doe 3 has had privileges at Christus “off and on” for “30 years” and was reappointed to the Christus Health staff in 2012 and again in 2014. App. 272; Record 12102 (2012–2014); *id.*, at 12112 (2014–2016). Throughout this time, he performed abortions. App. 206, 210.

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Attempting to justify Doe 2's decision not to (re)apply to Christus, the plurality suggests that Doe 3 (and by extension Doe 2) successfully concealed their abortion practice from Christus, and that if Doe 2 had applied for privileges, Christus would have discovered that he was performing abortions and denied his application on that ground. It is doubtful that Christus was actually in the dark, and speculative that an application would have been denied for this reason.¹⁰ But the important point is that a doctor with a strong

¹⁰The suggestion that Doe 2's abortion practice could have eluded Christus (and therefore that it would be an impediment to obtaining privileges again) blinks reality. There is no evidence that the hospital was unaware of Doe 2's abortion practice when he was on staff. Nor is there reason to believe that Christus would not have reviewed Doe 2's professional practice history, Record 12190–12191, or demanded disclosure of past malpractice claims at the time he held privileges there, *id.*, at 12194; App. 374 (medical malpractice claim against Doe 2 arising from practice at June Medical); see also *supra*, at 380–383 (reviewing hospital credentialing).

The notion that Doe 3's abortion practice has escaped attention for 30 years is even harder to believe. Christus has reappointed Doe 3 in recent years based on a biennial process that assesses “[p]erformance and conduct in each hospital and/or other healthcare organizatio[n]” outside of Christus. Record 12136; see also *ibid.* (requiring staff members to submit “reapplication form [with] complete information to update his/her file on items listed in his/her original application”). Doe 3 spends “Thursday afternoon” and “all day on Saturday” at the abortion clinic, App. 206, and therefore presumably is unavailable for his on-call duties at Christus at those times, Record 12123. Doe 3 is affiliated with the National Abortion Federation and has attended “many” of their national conferences to obtain continuing medical education credits. App. 203. And Doe 3 indicated that all eight OB/GYNs in Bossier City learned of his abortion practice when discussing a possible on-call rotation system. See *id.*, at 200–202. If those facts did not tip off the hospital, perhaps Christus learned about Doe 3's abortion practice when one of his patients was transferred directly from June Medical to Christus, bleeding and in need of a hysterectomy, *id.*, at 217–218, or when Doe 1's privileges application named Doe 3 as a peer reference, Record 13025. Whatever the Christus bylaws say, abortion practice does not appear to have presented an obstacle to a successful association with the hospital.

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personal incentive would have tried and not simply gone through the motions.

Instead of applying to Christus Health, Doe 2 made a formal application to Willis-Knighton Bossier City (WKBC) and an informal inquiry at University Hospital, but the record does not show that he pursued those requests with any zeal. At WKBC, he did not apply for courtesy privileges, which do not require a minimum number of admissions, Record 9642–9643, but instead sought an active staff position, *id.*, at 9751, and according to Doe 2, this application was doomed because he could not satisfy the minimum-admissions requirement for such a position, App. 384–390. Doe 2 later sent a three-paragraph e-mail to a WKBC e-mail address purporting to amend his 102-page application so as to seek only courtesy privileges, *id.*, at 1446, but the record does not reflect whether that e-mail was received or processed, and subsequent correspondence from WKBC does not acknowledge it, *id.*, at 1435. Doe 2 stated that he sought an active staff position “to keep [his] practice options for the future open,” Record 9756, but that does not explain his lack of diligence in seeking courtesy staff privileges. Although it is true that WKBC requested inpatient records from Doe 2 for an active staff position, we do not know whether the hospital would have made the same request had Doe 2 applied for courtesy privileges. App. 1435.¹¹

Doe 2 said he made an informal inquiry about admitting privileges at University Hospital, where he has consulting privileges, but that the head of the OB/GYN Department,

¹¹ Each year, a physician with courtesy staff privileges at WKBC may have as many as 49 “patient contacts,” which are defined as “any admission and management, consultation, procedure, response to emergency call, and newborns.” *Id.*, at 9628, 9642 (capitalization omitted). And contrary to the plurality’s suggestion, the fact that WKBC imposes the same “[f]actors for [e]valuation” for courtesy and active staff-applicants says little, since those factors do not set out any quantum of patient records, and require only “relevant . . . experience” for the position sought. *Id.*, at 9669.

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Dr. Groome, “essentially said” that the hospital would not upgrade his credentials. *Id.*, at 384. Doe 2 attributed this to “the political nature of what I do and the controversy of what I do.” *Ibid.* But Doe 2 did not introduce evidence (or seek to elicit testimony from Dr. Groome) substantiating his account of this informal inquiry.

Doe 2’s account raises obvious questions. Since he was *already* a member of the University Hospital staff, it is not apparent why the hospital would reject his request for upgraded privileges because of “the political nature” of his practice. *Id.*, at 440–441. And University Hospital has long been on notice of Doe 2’s abortion practice. He has been affiliated with that hospital since 1979, Record 9757, and has performed abortions since 1980, *id.*, at 9759.

In sum, Doe 2 all but admitted in his e-mails that his efforts to obtain privileges were perfunctory; he declined to apply at a hospital where he previously had privileges; at the only hospital where he made a formal application, he sought a position he knew he could not get for lack of a sufficient number of admissions; and at one other hospital (where he already had consulting privileges) he did no more than make an informal inquiry. The District Court should have considered whether Doe 2’s efforts were consistent with the conduct of a person who really wanted to get privileges.

Doe 5. Doe 5 is an OB/GYN who performs abortions at Women’s Clinic in New Orleans and Delta Clinic in Baton Rouge. Doe 5 did not testify at the hearing in District Court, but the District Court found that he proceeded in “good faith” based on a declaration and the transcript of a deposition. 250 F. Supp. 3d, at 75–76.

Doe 5 obtained courtesy privileges at Touro Hospital in New Orleans, see App. 1401, and therefore all agree that Act 620 would not prevent him from practicing at Women’s Clinic, *id.*, at 1397. The remaining question is whether the law would bar him from performing abortions in Baton Rouge.

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Doe 5 could continue to do that if one hospital in that area granted him admitting privileges, and Doe 5 testified that one, Woman's Hospital, will grant him privileges once he finds a doctor who is willing to cover him when he is not available. See *id.*, at 1334. Doe 5 asked exactly one doctor to serve as his covering physician. That does not show that he "could not find a covering physician," *ante*, at 328, if he made other inquiries.

The plurality justifies Doe 5's meager effort based on pure speculation. Because the one doctor Doe 5 asked had a transfer agreement with the Baton Rouge abortion clinic, the plurality reasons that "Doe 5 could have reasonably thought that, if this doctor wouldn't serve as his covering physician, no one would." *Ante*, at 332. The plurality goes on to say that "it was well within the District Court's discretion to credit that reading of the record." *Ibid.*

This argument shows how far the plurality is willing to go to strike down the Louisiana law. The plurality relies on speculation about why Doe 5 made only one inquiry and why the District Court found this one inquiry sufficient. In fact, however, Doe 5 never explained why he asked only one doctor, and he never intimated that he gave up because that doctor had a transfer agreement with the clinic. Nor did the District Court rely on that inference in finding that Doe 5 exhibited good faith. See 250 F. Supp. 3d, at 75–76. And in any event, even if Doe 5 had a particularly strong reason to hope that the doctor he asked would agree to cover for him, it hardly follows that other inquiries would necessarily fail.

Doe 5 applied for privileges at two other area hospitals, Lane and Baton Rouge, but he did not even call back to check on them because he thought his "best chances for privileges [were] at Woman's Hospital," App. 1334, and he noted that Lane and Baton Rouge require that their doctors treat some indigent patients "for free basically" while opening them-

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selves up to liability, *id.*, at 1335. Also, Doe 5 explained, Lane is “further away” from the Delta Clinic than the other hospitals. *Ibid.*

To sum up Doe 5’s situation: The challenged law would have no effect on him if he could find a covering doctor in Baton Rouge, but he asked only one doctor. He did little to pursue applications at two other hospitals because he was not optimistic about his chances and those hospitals required a certain amount of unpaid service to the poor.

Doe 6. Doe 6 is a Board-certified OB/GYN who practices at Women’s Clinic in New Orleans. There are nine qualifying New Orleans-area hospitals, and according to his affidavit, Doe 6 made an informal inquiry at one and filed a formal application at another. The District Court found that he attempted in “good faith” to obtain admitting privileges even though Doe 6 did not testify and was never subjected to adversarial questioning. The only relevant information before the court were several paragraphs in Doe 6’s declaration, *id.*, at 1307–1313, and hearsay in the declaration of the Women’s Clinic administrator, *id.*, at 1119–1131; see also 250 F. Supp. 3d, at 76–77.

These questionable sources left many important questions unanswered, for example, why Doe 6 did not apply for privileges at Touro Hospital, where Doe 5, who also performs abortions at Women’s Clinic, has privileges.

The plurality provides an explanation that is found nowhere in the record, *i.e.*, that Doe 6 could not get privileges at Touro because, unlike Doe 5, who performs both surgical and medication abortions, Doe 6 performs only medication abortions. *Ante*, at 335. Not only is this pure speculation, but it is not evident why this difference might matter. The plurality notes that Doe 6’s medication abortion patients have never been admitted to a hospital, but the plurality also argues that very few surgical abortion patients are admitted. *Ante*, at 334, 341. If Doe 6 had testified or been deposed, he

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could have been asked about his decision not to apply at Touro, but that did not occur.

Aside from Touro, there are eight other hospitals in the New Orleans area, but Doe 6 apparently made no attempt to get privileges at six of these, and nothing in the scant record explains why. He stated that he formally applied at East Jefferson Hospital and made an informal inquiry at Tulane Hospital, but much about these efforts is unknown. No representative from Tulane or East Jefferson testified or was deposed, and no documents relating to either application were offered.

With respect to Doe 6's informal inquiry at Tulane, all that the District Court had before it was a single paragraph in Doe 6's declaration in which he stated that he spoke to an unnamed individual and was told he should not bother to apply because he did not have the requisite number of admissions per year. App. 1310. Nothing in the record reveals the type of privileges about which Doe 6 inquired.

Doe 6 furnished even less information about his formal application to East Jefferson hospital—a hospital which offers courtesy privileges, and does not impose an admissions requirement for those privileges. Record 10679. In his declaration, which he signed in September 2014, Doe 6 wrote that he had applied but had not received a response. App. 1311. A few weeks later, June Medical's counsel informed the District Court by letter that Doe 6 had complied with East Jefferson's request for additional information, *id.*, at 54, but the record says nothing about any later developments. Presumably, East Jefferson did not grant privileges, but the record does not disclose why. Did Doe 6 provide all the information that the hospital requested and do everything else required by the application process? The record is silent, and the District Court was incurious.

Doe 3. Doe 3, who performs abortions at the June Medical clinic in Shreveport, would not be directly affected by Act

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620 because he maintains privileges at two area hospitals, Christus Health and WKBC, but he stated that he would stop performing abortions if, as a result of that law, he was left as the only abortion doctor in the northern part of the State. *Id.*, at 236. Thus, if Doe 1 or Doe 2 got privileges and continued to perform abortions, Doe 3, according to his testimony, would remain as well.¹²

Putting all this together, it is apparent that the record does not come close to showing that Doe 2, Doe 5, and Doe 6 made the sort of effort that one would expect if their ability to continue performing abortions had depended on success. These doctors had an incentive to do the bare minimum that they thought the judge would demand—and as it turned out, the judge did not demand much, not even an appearance in his courtroom. In short, the record does not show that Act 620 would drive any of these doctors out of abortion practice, and therefore the Act would not lead Doe 3 to leave either. It follows that the District Court’s finding on Act 620’s likely effects cannot stand.

C

The Court should remand this case for a new trial under the correct legal standards. The District Court should apply *Casey*’s “substantial obstacle” test, not the *Whole Woman’s Health* balancing test. And it should require those challenging Act 620 to demonstrate that the doctors who lack admitting privileges attempted to obtain them with the same zeal they would have exhibited if the Act were in effect and they stood to lose by failing in those efforts.

¹²The plurality suggests that, if Doe 3 were to leave abortion practice, it would be attributable to Act 620. But even the most ardent opponents of Act 620 did not contemplate that the law would prompt abortion doctors who *satisfied* the law’s requirements to quit. Record 11231–11234, 11291. And if this outcome was not foreseeable at the time of enactment, it is hard to see how the District Court could blame Act 620 for causing Doe 3 to leave abortion practice. Cf. Restatement (Second) of Torts § 440, § 442A (1964).

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IV

On remand, the District Court should not permit June Medical to assert the rights of women wishing to obtain an abortion. The court should require the joinder of a plaintiff whose own rights are at stake. Our precedents rarely permit a plaintiff to assert the rights of a third party, and June Medical cannot satisfy our established test for third-party standing. Indeed, what June Medical seeks is something we have never allowed. It wants to rely on the rights of third parties whose interests conflict with its own.

A

The plurality holds that Louisiana waived any objection to June Medical's third-party standing, *ante*, at 316, but that is a misreading of the record. The plurality relies on a passing statement in a brief filed by the State in District Court in connection with the plaintiffs' request for a temporary restraining order, but the statement is simply an accurate statement of Circuit precedent on the standing of abortion providers. See App. 44. It does not constitute a waiver.

It is true that Louisiana did not affirmatively make the third-party standing argument until it filed its cross-petition for certiorari, but "[w]e may make exceptions to our general approach to claims not raised below." *Polar Tankers, Inc. v. City of Valdez*, 557 U. S. 1, 14 (2009) (plurality opinion). A party's failure to raise an issue does not deprive us of the power to take it up, so long as the court below has passed on the question. See *Lebron v. National Railroad Passenger Corporation*, 513 U. S. 374, 379 (1995) ("[E]ven if this were a claim not raised by petitioner below, we would ordinarily feel free to address it, since it was addressed by the court below" (emphasis deleted)); S. Shapiro et al., *Supreme Court Practice* § 6–26(b), p. 6–104 (11th ed. 2019) (collecting cases).

In this case, no one disputes that the Fifth Circuit passed on the issue of third-party standing in Louisiana's appeal from the District Court's entry of a preliminary injunction.

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June Medical Services, L. L. C. v. Gee, 814 F. 3d 319, 322–323 (2016). And when we granted the State’s cross-petition, we took up this question and received briefing and argument on it. 588 U. S. 948 (2019).

We have a strong reason to decide the question of third-party standing because it implicates the integrity of future proceedings that should occur in this case. This case should be remanded for a new trial, and we should not allow that to occur without a proper plaintiff. Nothing compels us to forbear from addressing this issue. See *Carlson v. Green*, 446 U. S. 14, 17, n. 2 (1980); Shapiro, *Supreme Court Practice* § 6.26(h), at 6–111.

B

This case features a blatant conflict of interest between an abortion provider and its patients. Like any other regulated entity, an abortion provider has a financial interest in avoiding burdensome regulations such as Act 620’s admitting-privileges requirement. Applying for privileges takes time and energy, and maintaining privileges may impose additional burdens. See App. 1335. Women seeking abortions, on the other hand, have an interest in the preservation of regulations that protect their health. The conflict inherent in such a situation is glaring.

Some may not see the conflict in this case because they are convinced that the admitting-privileges requirement does nothing to promote safety and is really just a ploy. But an abortion provider’s ability to assert the rights of women when it challenges ostensible safety regulations should not turn on the merits of its claim.

The problem with the rule that the plurality embraces is highlighted if we consider challenges to other safety regulations. Suppose, for example, that a clinic in a State that allows certified non-physicians to perform abortions claims that the State’s certification requirements are too onerous and that they imperil the clinic’s continued operation. Should the clinic be able to assert the rights of women in

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attacking this regulation, which the state lawmakers thought was important to protect women's health?

When an abortion regulation is enacted for the asserted purpose of protecting the health of women, an abortion provider seeking to strike down that law should not be able to rely on the constitutional rights of women. Like any other party unhappy with burdensome regulation, the provider should be limited to its own rights.

C

This rule is supported by precedent and follows from general principles regarding conflicts of interest. We have already held that third-party standing is not appropriate where there is a potential conflict of interest between the plaintiff and the third party. In *Elk Grove Unified School Dist. v. Newdow*, 542 U. S. 1, 9, 15, and n. 7 (2004), a potential conflict of interest between the plaintiff and his daughter arose on appeal. The father had asserted that his daughter had a constitutional right not to hear others recite the words “‘under God’” when the pledge of allegiance was recited at her public school, but the child's mother maintained that her daughter had “no objection either to reciting or hearing” the full pledge. *Id.*, at 5, 9. The Court held that the father lacked prudential standing, because “the interests of this parent and this child are not parallel and, indeed, are potentially in conflict.” *Id.*, at 15. The lower court's judgment (based, as it was, on a presentation by a conflicted party) was therefore reversed.

Newdow recognized the seriousness of conflicts of interest in the specific context of third-party claims, but the law is always sensitive to potential conflicts when a party sues in a representative capacity. Parties naturally “tailor their own presentation to the interest that each of them has,” and a conflict therefore creates “a risk that the party will not provide adequate representation of the interest of the absentee.” See 7C Wright & Miller § 1909 (2007). Thus, in class-action

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suits, Federal Rule of Civil Procedure 23(a)(4) demands that the named plaintiff possess “the same interest and suffer the same injury” as class members. *General Telephone Co. of Southwest v. Falcon*, 457 U. S. 147, 156 (1982) (internal quotation marks omitted). That requirement, we have said, “serves to uncover conflicts of interest between named parties and the class they seek to represent.” *Amchem Products, Inc. v. Windsor*, 521 U. S. 591, 625 (1997). Similarly, under Federal Rule of Civil Procedure 17(c), a party representing a minor or incompetent person may be replaced if the representative has conflicting interests. See *Sam M. v. Carcieri*, 608 F. 3d 77, 86 (CA1 2010); 6A Wright & Miller § 1570 (2010). And of course, an attorney cannot represent a client if their interests conflict.¹³

D

The conflict of interest inherent in a case like this is reason enough to reject third-party standing, and our standard rules on third-party standing provide a second, independent reason. As a general rule, a plaintiff “must assert his own legal rights and interests, and cannot rest his claim to relief on the legal rights or interests of third parties.” *Warth v. Seldin*, 422 U. S. 490, 499 (1975). We have recognized a “limited” exception to this rule, but in order to qualify, a litigant must demonstrate (1) closeness to the third party and (2) a hindrance to the third party’s ability to bring suit. *Kowalski v. Tesmer*, 543 U. S. 125, 129–130 (2004); see also *Powers v. Ohio*, 499 U. S. 400, 410–411 (1991).

The record shows that abortion providers cannot satisfy either prong of this test. First, a woman who obtains an abortion typically does not develop a close relationship with the doctor who performs the procedure. On the contrary, their relationship is generally brief and very limited. In Louisiana, a woman may make her first visit to an abortion

¹³ See, e. g., ABA Model Rules of Professional Conduct 1.7–1.9, 1.18 (2016).

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clinic the day before the procedure, and if she goes to June Medical, she is likely to have a short meeting with a counselor, not the doctor who will actually perform the procedure. See App. 784–786. She will typically meet the abortion doctor for the first time just before the procedure, and if Doe 1’s description is representative, their relationship consists of the doctor’s telling the woman what he will do, offering to answer questions, informing her of his progress as the abortion is performed, and asking her to remain calm. *Id.*, at 688. Doe 4 testified that the surgical procedure itself takes “two or three minutes.” Record 14144. Doe 3 testified that he can perform six abortions an hour and once performed 64 abortions in a 2-day period. App. 207, 243.

In the case of medication abortions, patients are required to schedule a follow-up appointment three weeks after the procedure, see *id.*, at 129–131, 690, but surgical abortions, which constitute the majority of the procedures at June Medical and across the State, do not require any follow-up, *id.*, at 691, and the great majority of women never return to the clinic, *id.*, at 131; accord, *id.*, at 1342 (Doe 5).

This description of doctor-patient interactions at June Medical is similar to those recounted in testimony heard by the legislature. See Record 11263 (“there was no doctor/patient relationship”); *id.*, at 11226 (“I can tell you, women I’ve counseled, many times they don’t know who the abortion provider is”). *Amici* who have had abortions recount similarly distant relationships with their abortion doctors.¹⁴ For these reasons, the first prong of the third-party standing rule cannot be met.

Nor can the second, which requires that there be a hindrance to the ability of the third party to bring suit. See *Kowalski*, 543 U. S., at 130. The plurality opinion in *Single-*

¹⁴ See Brief for 2,624 Women Injured by Abortion et al. as *Amici Curiae* 14–22 (firsthand accounts of abortion procedures in Louisiana); Brief for Priests for Life et al. as *Amici Curiae* 7–8, and App. (accounts from Louisiana and other States).

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ton v. Wulff, 428 U.S. 106, 117 (1976), found that women seeking abortions were hindered from bringing suit, but the reasoning in that opinion is hard to defend. The opinion identified two purported obstacles to suits by women wishing to obtain abortions—the women’s desire to protect their privacy and the prospect of mootness. *Ibid.* But as Justice Powell said at the time, these “alleged ‘obstacles’ . . . are chimerical.” *Id.*, at 126 (opinion concurring in part and dissenting in part).

First, a woman who challenges an abortion restriction can sue under a pseudonym, and many have done so. *Ibid.* (“Our docket regularly contains cases in which women, using pseudonyms, challenge statutes that allegedly infringe their right to exercise the abortion decision”). Other precautions may be taken during the course of litigation to avoid revealing their identities. See App. 196.¹⁵ And there is little reason to think that a woman who challenges an abortion restriction will have to pay for counsel. See Brief for Respondent/Cross-Petitioner 40–41.

Second, if a woman seeking an abortion brings suit, her claim will survive the end of her pregnancy under the capable-of-repetition-yet-evading-review exception to mootness. See *Roe v. Wade*, 410 U.S. 113, 125 (1973) (“Pregnancy provides a classic justification for a conclusion of non-

¹⁵ Four cases to reach this Court have featured exclusively women plaintiffs. See *Beal v. Doe*, 432 U.S. 438 (1977); *Maher v. Roe*, 432 U.S. 464 (1977); *Poelker v. Doe*, 432 U.S. 519 (1977) (*per curiam*); *H. L. v. Matheson*, 450 U.S. 398 (1981). But there are a number of cases in which women have been co-plaintiffs along with abortion clinics or providers. See *Leavitt v. Jane L.*, 518 U.S. 137 (1996) (*per curiam*); *Ohio v. Akron Center for Reproductive Health*, 497 U.S. 502 (1990); *Hodgson v. Minnesota*, 497 U.S. 417 (1990); *Williams v. Zbaraz*, 448 U.S. 358 (1980); *Harris v. McRae*, 448 U.S. 297 (1980); *Bellotti v. Baird*, 443 U.S. 622 (1979); *Roe v. Wade*, 410 U.S. 113 (1973). More recently, abortion patients have litigated in the lower courts using their names, those of legal guardians, or pseudonyms. Brief for Respondent/Cross-Petitioner 39; see also Brief for State of Arkansas et al. as *Amici Curiae* 3, and n. 1.

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mootness”). To be sure, when the pregnancy terminates, an individual plaintiff’s immediate interest in prosecuting the case may diminish. But this is generally true whenever the capable-of-repetition-yet-evading-review exception applies. See 13C Wright & Miller § 3533.8 (2008) (collecting examples).

The *Singleton* plurality opinion is the only opinion in which any Members of this Court have ever attempted to justify third-party standing for abortion providers, and judged on its own merits, the opinion is thoroughly unconvincing.

E

The Court does not address the conflict of interest inherent in this challenge, or plaintiffs’ failure to satisfy the two prongs of our third-party standing doctrine. See *Kowalski*, 543 U. S., at 130. Instead, the plurality says that it “is . . . common” in third-party standing case law for “plaintiffs [to] challeng[e] a law ostensibly enacted to protect [a third party] whose rights they are asserting.” *Ante*, at 320. In support of this strange proposition, the plurality cites two of our prior decisions, but neither decision acknowledged or addressed any potential conflict of interest, and both cases involved circumstances very different from those present here. Both cases also featured facts ensuring that third-party interests were fairly represented.

In the first case, *Craig v. Boren*, 429 U. S. 190 (1976), the sole appellant with a live claim at the time of decision was a beer vendor who challenged a law that allowed females to purchase 3.2% beer at the age of 18 but barred males from making such purchases until they turned 21. *Id.*, at 193. The Court’s lead explanation for its refusal to dismiss had nothing to do with the merits of the vendor’s third-party standing claim. The Court noted that the other appellant, Curtis Craig, had been under the age of 21 during the proceedings below, that the appellees had not raised a standing objection below, and that they had not pressed an objection in this Court. *Id.*, at 192–194.

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Only after this discussion did the Court say anything about the merits of the third-party claim, and even then, the Court said nothing about a conflict of interest between the vendor and underage males. The plurality now claims there was a potential conflict: Young men under the age of 21 had an interest in being barred from buying beer in order to protect themselves from their own reckless conduct. Suffice it to say that there is no indication that this supposed conflict occurred to anybody when *Craig* was before this Court.

The plurality's second case, *Department of Labor v. Triplett*, 494 U. S. 715 (1990), is even weaker. A state bar ethics committee filed a disciplinary proceeding in state court against a lawyer who had entered into an attorney-fee arrangement that was prohibited by a provision of the Black Lung Benefits Act. When the State Supreme Court ruled in favor of the lawyer on the ground that the provision in question violated Black Lung claimants' constitutional right to counsel, both the bar ethics committee and the Department of Labor, which had intervened in state court, successfully petitioned for review in this Court. We then held that the attorney could defend the decision below based on the rights of his client.

Triplett is inapposite here for at least two reasons. First, the lawyer in that case did not initiate the litigation. Second, because the case arose in state court, his right to invoke his client's rights in that forum was a question of state law. Had we prevented him from asserting those rights in this Court, he would have been unable to defend himself against the petitioners' arguments. And on top of all this, *Triplett*, as we noted in *Kowalski*, "involved the representation of known claimants," and that "existing attorney-client relationship [was] quite different from the hypothetical . . . relationship" between the abortion providers and clients in the present case. 543 U. S., at 131 (emphasis deleted). That *Craig* and *Triplett* are the best authorities the plurality can find is telling proof of the weakness of its position.

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F

As THE CHIEF JUSTICE points out, *stare decisis* generally counsels adherence to precedent, and in deciding whether to overrule a prior decision, we consider factors beyond the strength of the precedent's reasoning. *Ante*, at 346. But here, such factors weigh in favor of overruling.

Reexamination of a precedent may be appropriate when it is an “outlier” and its reasoning cannot be reconciled with other established precedents, see *Franchise Tax Bd. of Cal. v. Hyatt*, 587 U. S. 230, 248 (2019); *Janus v. State, County, and Municipal Employees*, 585 U. S. 878, 925 (2018); *United States v. Gaudin*, 515 U. S. 506, 521 (1995); *Rodriguez de Quijas v. Shearson/American Express, Inc.*, 490 U. S. 477, 484 (1989), and that is true of the rule allowing abortion providers to assert their patients' rights. The parties have not brought to our attention any other situation in which a party is allowed to invoke the right of a third party with blatantly adverse interests. The rule that the majority applies here is an abortion-only rule.

THE CHIEF JUSTICE properly notes that subsequent legal developments may support overruling a precedent, *ante*, at 346, and that factor too is present here. Both our general standing jurisprudence and our treatment of third-party standing have changed since *Singleton*. We have stressed the importance of insisting that a plaintiff assert an injury that is particular to its own situation. See, e. g., *Spokeo, Inc. v. Robins*, 578 U. S. 330, 339 (2016); *Clapper v. Amnesty Int'l USA*, 568 U. S. 398, 409 (2013); *Lujan v. Defenders of Wildlife*, 504 U. S. 555, 560 (1992). Moreover, in *Kowalski*, 543 U. S. 125, we refined our rule for third-party standing, and in *Newdow*, 542 U. S. 1, we made it clear that a plaintiff cannot sue on behalf of a third party if the parties' interests may conflict.

The presence or absence of reliance is often a critical factor in applying the doctrine of *stare decisis*, see, e. g., *Franchise Tax Bd.*, 587 U. S., at 248; *Janus*, 585 U. S., at 926;

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South Dakota v. Wayfair, Inc., 585 U. S. 162, 186 (2018); *Hilton v. South Carolina Public Railways Comm’n*, 502 U. S. 197, 206–207 (1991), but neither the plurality nor THE CHIEF JUSTICE claims that any reliance interests are at stake here. Women wishing to obtain abortions have not taken any action in reliance on the ability of abortion providers to sue on their behalf, and eliminating third-party standing for providers would not interfere with the ability of women to sue. Nor does it appear that abortion providers have done anything in reliance on the special third-party standing rule they have enjoyed. If that rule were abrogated, they could still ask to intervene or appear as an *amicus curiae* in a suit brought by a woman, but it is deeply offensive to our rules of standing to permit them to sue in the name of their patients when they challenge laws enacted to protect their patients’ safety.

On remand, the District Court should permit the joinder of a plaintiff with standing and should not proceed until such a plaintiff appears.

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The decision in this case, like that in *Whole Woman’s Health*, twists the law, and I therefore respectfully dissent.

JUSTICE GORSUCH, dissenting.

The judicial power is constrained by an array of rules. Rules about the deference due the legislative process, the standing of the parties before us, the use of facial challenges to invalidate democratically enacted statutes, and the award of prospective relief. Still more rules seek to ensure that any legal tests judges may devise are capable of neutral and principled administration. Individually, these rules may seem prosaic. But, collectively, they help keep us in our constitutionally assigned lane, sure that we are in the business of saying what the law is, not what we wish it to be.

Today’s decision doesn’t just overlook one of these rules. It overlooks one after another. And it does so in a case

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touching on one of the most controversial topics in contemporary politics and law, exactly the context where this Court should be leaning most heavily on the rules of the judicial process. In truth, *Roe v. Wade*, 410 U. S. 113 (1973), is not even at issue here. The real question we face concerns our willingness to follow the traditional constraints of the judicial process when a case touching on abortion enters the courtroom.

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When confronting a constitutional challenge to a law, this Court ordinarily reviews the legislature's factual findings under a "deferential" if not "[u]ncritical" standard. *Gonzales v. Carhart*, 550 U. S. 124, 165–166 (2007). When facing such a challenge, too, this Court usually accepts that "the public interest has been declared in terms well-nigh conclusive" by the legislature's adoption of the law—so we may review the law only for its constitutionality, not its wisdom. *Berman v. Parker*, 348 U. S. 26, 32 (1954). Today, however, the plurality declares that the law before us holds no benefits for the public and bears too many social costs. All while sharing virtually nothing about the facts that led the legislature to conclude otherwise. The law might as well have fallen from the sky.

Of course, that's hardly the case. In Act 620, Louisiana's legislature found that requiring abortion providers to hold admitting privileges at a hospital within 30 miles of the clinic where they perform abortions would serve the public interest by protecting women's health and safety. Those in today's majority never bother to say so, but it turns out that Act 620's admitting privileges requirement for abortion providers tracks longstanding state laws governing physicians who perform relatively low-risk procedures like colonoscopies, Lasik eye surgeries, and steroid injections at ambulatory surgical centers. In fact, the Louisiana legislature passed Act 620 only after extensive hearings at which ex-